A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

29 April 2026

A/Prof Michael Hii
Suite 301, Level 3

Dear A/Prof Hii,

Re: Ms Lara Mainka
DOB: 14/10/1968 Phone: 0422 340 865
Address: 4 Vernon Street, UPPER FERNTREE GULLY, VIC, 3156
Email: hullara@yahoo.com.au
Medicare: 3331209988 / 1

Summary

- **Hypercontractile oesophagus**
- **Improvement in abdominal pain, bloating, and pelvic floor symptoms secondary to physiotherapy and amitriptyline**
- **Gastroscopy and colonoscopy findings unremarkable with biopsies normal**
- **Trial of low-dose nifedipine and titration of amitriptyline.**
- **No action required by you.**

I had the pleasure of conducting a review with Lara to discuss the results of her recent investigations. While the macroscopic findings of her gastroscopy were unremarkable, showing no evidence of malignancy, inflammatory bowel disease, or macroscopic colitis. The most significant finding arises from her pH manometry, which confirmed a hypercontractile oesophagus. This diagnosis explains her symptoms of dysphagia, particularly the sensation of tablets becoming lodged, as well as her reflux. We discussed the potential aetiology, noting that while medications such as opiates can induce this motility pattern, Lara is not currently taking any. We reviewed her current regimen, including pregabalin and Maxalt, though these are less likely to be the primary drivers.

Lara reports a pleasing improvement in several of her other symptoms. Her abdominal pain and bloating have diminished since starting amitriptyline, and she is also seeing progress with her pelvic floor symptoms under the guidance of her excellent physiotherapist Isabella Lees Trinca.

Impression and Plan

The primary challenge in managing Lara's hypercontractile oesophagus is the crossover between necessary pharmacological intervention and her existing hypotension. Calcium channel blockers, such as nifedipine, are the first-line treatment to relax the oesophageal smooth muscle; however, these carry a high risk of exacerbating her lightheadedness, for which she already takes midodrine.

We have decided to proceed with a very cautious trial of low-dose nifedipine. I have advised Lara to initiate this over a weekend when she has support at home and to monitor her blood pressure closely. Furthermore, I have provided prescriptions for 10 mg and 25 mg amitriptyline to allow her to titrate her dose up to 35 mg or

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50 mg as tolerated, which should further assist with her abdominal symptoms.

If this conservative medical approach proves unsuccessful or intolerable due to side effects, we may consider more invasive options in the future, such as botulinum toxin injections into the oesophageal muscle or a peroral endoscopic myotomy. I have arranged to review Lara in three months' time to assess her progress.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Anjila Kant, Ms Isabella Lees-Trinca

All appointments and enquires

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Endoscopy locations

Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065